

Scoring the vendor questionnaires

Sixteen vendors have returned the questionnaire. Two ways to get from those to a shortlist — the same judgement, at two levels of depth.

1

The full scorecard

ABOUT 10 MINUTES A VENDOR

Each questionnaire is scored out of a hundred across the five things we said mattered. The team picks from menus; the sheet does the arithmetic.

25	Home Care Home Base integration	Do they already connect to the system our plans of care, orders and visits live in? Ticked from a list of six, not judged.
30	Scope coverage	How much of the scope we published they actually cover — reported separately for capacity, scheduling and engagement.
20	Product quality	Whether they explained how the product decides, or only that it exists.
10	Clinician fit	What the clinician still controls, and whether adoption is proven rather than promised.
15	Partnership	Whether they will trade on the investment we are making.

Home Care Home Base is a quarter of the score and a **tick box rather than a judgement**, so the priority cannot get quietly watered down as we work through sixteen vendors.

Scope is measured against the one-pager we published, so each vendor is compared to **what we asked for** rather than to one another's marketing. Everywhere else, the score rewards a vendor who showed us how their product decides over one who simply says it does.

What comes out is a number we can defend line by line, plus three short lists for each vendor: what makes them different, what worries us, and what they did not answer.

2

The simple scorecard

ABOUT 3 MINUTES A VENDOR

The same judgement at a coarser grain — twelve rows a vendor instead of fifty.

Home Care Home Base	Picked from the same list of six.
Eleven areas	The ones each vendor already rated themselves on in the questionnaire.
Two marks each	What they have, and how good it is out of a hundred.
It adds up	A coverage percentage and a rating for capacity, scheduling and engagement.

It follows the functional scorecard already in our workbook, so it **reads like something the team has seen before**. Most of it is transcription rather than judgement: what a vendor has comes straight from their own answer.

This is the fast read across the field. What it cannot do is separate two vendors who both cover an area but do it very differently, and it carries no partnership or clinician score at all — so **where the shortlist is close, option one is the one that holds up**.